

THE INDIAN FAT-LOSS PLAYBOOK

Why Indians Lose Weight Differently

Science-backed. Desi-food first. Built for India's body type.

30.7% WOMEN OVERWEIGHT NFHS-6, 2023-24, up from 24%	1.2-1.5g PROTEIN / KG target during fat loss	12 WEEKS the complete plan
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Read This First

This book is a system, not a diet. It gives you the numbers, the food, and the routine that actually work for an Indian body, Indian kitchens, and an Indian calendar full of festivals. Every statistic is drawn from official Indian and international sources, listed at the back.

A word on health. This is general education, not medical advice. Weight, thyroid, PCOS, diabetes and heart health are personal. If you are pregnant or breastfeeding, under 18, managing a chronic condition, on medication, or planning to use a weight-loss medicine, speak to a qualified doctor before changing how you eat or move. Nothing here replaces a diagnosis or a prescription.

How to use it. Read Chapters 1 to 4 to understand why your body behaves the way it does, then go straight to Chapter 17 and start the 12-week plan. Use Chapters 5 to 16 as your reference library whenever you hit a wall, a festival, or a plateau.

The promise, in one line

Eat real Indian food, in the right amounts, with enough protein, and move a little more than you do today. Do it for twelve weeks and the fat comes off. No powders, no starvation, no imported 'superfoods'.

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CHAPTER 1

The Indian Weight Problem, in Numbers

India is getting heavier, faster than almost anyone expected, and the official data proves it. The National Family Health Survey 6 (NFHS-6), run by the Union Health Ministry and released in May 2026, is the country's most authoritative health snapshot. Its verdict is blunt.

30.7%**WOMEN 15-49**

overweight or obese, up from 24.0% in 2019-21

27.3%**MEN 15-49**

overweight or obese, up from 22.9%

42.8%**URBAN WOMEN**

overweight or obese, versus 25.5% rural

That is a jump of 6.7 percentage points for women and 4.4 for men in just five years. Nearly one in three Indian women and more than one in four men aged 15-49 now carries excess weight. Blood sugar tells the same story: high or very high blood sugar, or diabetes medication, rose from 13.5% to 17.8% among women and from 15.6% to 20.9% among men.

The ICMR-INDIAB study, published in The Lancet Diabetes and Endocrinology and covering 113,043 people across 31 states and union territories, found diabetes in 11.4% of adults, prediabetes in 15.3%, generalised obesity in 28.6%, and abdominal obesity in 39.5%. Its 2021 projections put 101 million Indians with diabetes, 136 million with prediabetes, and 351 million with abdominal obesity.

Why this matters to you

These are not numbers about 'other people'. They are the statistical fingerprint of the environment you live in: refined-carb-heavy meals, long sitting hours, poor sleep, and a festival calendar that never really ends. Fat gain in India is a system problem. This book is your personal fix.

The good news

Body fat is not destiny. The same official guidelines that describe the problem also describe the fix: enough protein, less refined carbohydrate, more fibre, less added sugar and oil, and regular movement. Every one of those levers is free or nearly free, and every one of them works on Indian food.

CHAPTER 2

Why Indians Gain Weight Differently

You are not imagining it. An Indian body can look 'normal' on the scale and still carry dangerous fat. The medical name for this is the Asian Indian phenotype, and understanding it is the single most useful thing in this book.

Four features define it:

- 1 **More visceral fat at the same weight.** Indians store more fat deep inside the abdomen (around the organs) rather than under the skin. Visceral fat is the metabolically dangerous kind: it drives insulin resistance, fatty liver and heart risk.
- 2 **Lower muscle mass.** Compared with Western populations, Indians typically start with less skeletal muscle. Less muscle means a lower resting burn and a smaller 'buffer' to absorb a calorie surplus.
- 3 **Earlier metabolic breakdown.** Diabetes and heart disease arrive at lower body-mass index (BMI) values. This is why Indian health bodies use stricter BMI and waist cutoffs (Chapter 4).
- 4 **The 'thin-fat' trap.** A person can have a normal BMI and still have central obesity and insulin resistance. ICMR-INDIAB found abdominal obesity in 39.5% of adults even as generalised obesity sat at 28.6%.

What this changes for you

It means three things. First, the scale is a poor referee, so you will track your waist as well. Second, protecting muscle with protein and strength work matters more for you than for almost anyone else. Third, you can get a real metabolic win without ever becoming 'thin' by Western standards, simply by cutting visceral fat and protecting muscle.

The one-sentence version

Your goal is not just to weigh less. It is to carry less dangerous fat and more muscle than you do today. That is the target this book aims at.

CHAPTER 3

The One Rule That Decides Everything: The Calorie Deficit

Every weight-loss method that has ever worked, from keto to intermittent fasting to a high-protein Indian plan, works for one reason: it puts you in a calorie deficit. Get this idea right and the rest is engineering. Ignore it and no superfood will save you.

Your body burns a number of calories each day called your Total Daily Energy Expenditure (TDEE). Eat fewer than that and you lose fat; eat more and you gain it. One kilogram of body fat holds roughly 7,700 kcal, so a daily deficit of about 500 kcal produces roughly 0.5 kg of fat loss per week. That is the sweet spot: fast enough to stay motivated, slow enough to protect muscle.

Finding your numbers

Start with your Basal Metabolic Rate (BMR), the energy you burn at complete rest. The Mifflin-St Jeor equation is the standard used in clinics:

Mifflin-St Jeor BMR

Men: $BMR = (10 \times \text{weight in kg}) + (6.25 \times \text{height in cm}) - (5 \times \text{age}) + 5$

Women: $BMR = (10 \times \text{weight in kg}) + (6.25 \times \text{height in cm}) - (5 \times \text{age}) - 161$

Multiply BMR by an activity factor:

Activity level	Multiplier	What it looks like
Sedentary	1.20	Desk job, little walking
Lightly active	1.375	Light exercise 1-3 days/week
Moderately active	1.55	Exercise 3-5 days/week
Very active	1.725	Hard exercise 6-7 days/week

Two worked examples

Person	BMR	Sedentary TDEE	Daily target (-500)
Woman, 30y, 70 kg, 160 cm	1,389 kcal	1,667 kcal	~1,170-1,270 kcal
Man, 32y, 80 kg, 175 cm	1,739 kcal	2,087 kcal	~1,590-1,690 kcal

Do not go below the floor. As a rule of thumb, keep daily intake at or above about 1,200 kcal for women and 1,500 kcal for men unless a doctor or dietitian directs otherwise. Crash dieting below this cuts muscle, wrecks your energy and rebounds. A smaller, sustainable deficit beats a brutal one every time.

CHAPTER 4

Your BMI Is Lying to You: Measure Your Waist Instead

BMI divides weight by height. It is a decent population tool and a poor personal one, especially for Indians. Two people with the same BMI can carry wildly different amounts of fat and risk. Worse, the standard BMI cutoffs were built on Western bodies and under-detect risk in Indians.

Indian and WHO Asia-Pacific cutoffs are stricter for a reason:

Category	Indian / Asia-Pacific BMI	Standard Western BMI
Normal	18.5 - 22.9	18.5 - 24.9
Overweight	23.0 - 24.9	25.0 - 29.9
Obese	25.0 and above	30.0 and above

So an Indian at BMI 24 is already 'overweight' by the measure that fits our bodies, and 25 counts as obese. If you have been reassured by a 'normal' BMI of 24, that reassurance was built on the wrong chart.

The measurement that matters more

Waist circumference is a far better gauge of the visceral fat that drives disease. Measure at the midpoint between the top of your hip bone and the bottom of your ribs, breathing out normally, tape snug but not compressing the skin. Indian action levels:

Measure	Men	Women	Meaning
Waist, action level 1	78 cm	72 cm	Start paying attention
Waist, action level 2	90 cm	80 cm	High cardiometabolic risk
Waist-to-hip ratio	0.90	0.85	Central fat flag
Waist-to-height ratio	0.50	0.50	Keep waist under half your height

Your two numbers

Forget a single weight reading. Track two things weekly: your weight (same day, morning, after the toilet, before food) and your waist in centimetres. If your weight is stuck but your waist is falling, you are winning: you are losing fat and holding muscle.

CHAPTER 5

The Protein Gap: India's Biggest Diet Mistake

If this book had to change only one thing about how you eat, it would be protein. The typical Indian plate is heavy on rice, roti and potato and light on protein, and that is the single biggest reason fat loss stalls and muscle slips away.

What the official numbers say

ICMR-NIN, the government's own nutrition institute, sets the protein Recommended Dietary Allowance for healthy Indian adults at 0.83 g per kg of body weight per day. For people eating a cereal-based diet with lower-quality protein, it rises to about 1 g per kg. For a 65 kg woman that is 54-65 g of protein a day, and most Indians eat well below even that.

0.83 g

RDA / KG / DAY

ICMR-NIN for healthy Indian adults

1.0 g

RDA / KG / DAY

for cereal-based diets

1.2-1.5 g

FAT-LOSS TARGET

per kg during active weight loss

During a fat-loss phase, leading Indian endocrinologists, in a 2026 consensus published in Obesity Pillars, recommend about 1.2 to 1.5 g of protein per kg per day to protect muscle, and at least 0.8 g per kg for maintenance. That is the target this book uses. For a 70 kg person, that is roughly 84 to 105 g of protein a day.

Why protein does so much work

- It protects muscle, so the weight you lose is fat, not the muscle that keeps your metabolism up.
- It is the most filling macronutrient, so you eat less without counting every bite.
- Digesting protein itself burns 20-30% of its calories, versus 6-8% for carbohydrate.
- It stabilises blood sugar, which matters enormously for Indians with prediabetes.

Quality, not just quantity

ICMR-NIN advises getting protein from a combination of foods, and combining cereals with pulses in roughly a 3:1 ratio to improve protein quality. It also explicitly advises against protein supplements for building muscle in normal, healthy people. Food first, always.

CHAPTER 6

The Indian Plate Method

You do not need a food scale or an app to eat right. You need a template. The Indian plate method rebuilds the thali so that protein and vegetables lead and carbohydrate follows, cutting 300-500 calories a day with no counting at all.

The new thali

Portion of the plate	Fill it with	Examples
One half	Vegetables and salad	Sabzi, cooked greens, cucumber, tomato, onion, kachumber
One quarter	Protein	Dal, rajma, chole, paneer, curd, eggs, chicken, fish, soya
One quarter	Smart carbohydrate	1-2 rotis, or one small bowl of rice, or millet

Notice what changed: rice and roti did not disappear, they simply stopped being the whole meal. This single swap, done twice a day, is often enough to create the deficit on its own.

The ICMR-NIN plate, in numbers

The government's Dietary Guidelines for Indians 2024 give concrete ceilings worth knowing: not more than about 45% of calories from cereals and millets; up to about 15% from pulses, beans and meat; added sugar under 5% of calories; and total fat at or under 30% of calories. It also recommends limiting salt and ultra-processed foods.

Build meals around these anchors

- Add a protein source to every meal, including breakfast. Eggs, besan chilla, paneer, curd, sprouts or a scoop of dal.
- Swap some refined grain for a whole one: millets (ragi, bajra, jowar), brown rice, whole-wheat atta, or dalia.
- Double the vegetables and keep a raw salad at lunch and dinner.
- Cook with less oil. The Prime Minister has publicly urged Indians to cut daily oil use by about 10%; that alone removes real calories.
- Finish meals with curd or a fruit instead of a sweet.

The 3:1 rule

ICMR-NIN recommends combining cereals with pulses in a 3:1 ratio to improve protein quality. Practically: for every three parts rice or roti, aim for one part dal, rajma or chole. This is how vegetarian Indian food delivers complete protein.

CHAPTER 7

Your Indian Food Calorie and Protein Tables

You cannot manage what you cannot see. These are practical calorie and protein values for common Indian foods, based on the ICMR-NIN Indian Food Composition Tables (IFCT 2017) and standard home portions. Treat them as decision tools, not lab reports: your oil, ghee and portion size move the numbers.

Protein per rupee: the foods that do the work

Food (common portion)	Energy	Protein	Note
Egg, 1 large	~70 kcal	~6 g	Cheapest complete protein
Curd / dahi, 1 cup (200 g)	~120 kcal	~6 g	Add to every meal
Soya chunks, 30 g dry	~105 kcal	~16 g	Highest plant protein
Paneer, 100 g	~265 kcal	~18 g	Watch the fat and calories
Toor / arhar dal, 1 cup cooked	~175 kcal	~10 g	The everyday staple
Chicken breast, 100 g	~165 kcal	~31 g	Lean and filling
Milk, 1 glass (200 ml)	~120 kcal	~6 g	Toned keeps calories lower
Rajma / chole, 1 cup cooked	~120 kcal	~8 g	Fibre plus protein

Carbohydrates: pick your portion

Food	Energy	Protein	Comment
Roti, 1 medium (40 g atta)	~114 kcal	~3 g	1-2 per meal is plenty
Cooked rice, 1 cup (150 g)	~195 kcal	~4 g	Portion, don't ban
Idli, 2 (80 g)	~77 kcal	~2 g	Low-calorie breakfast base
Poha, 1 bowl (150 g)	~250 kcal	~4 g	Watch the oil and peanuts
Banana, 1 medium	~107 kcal	~1 g	Good pre-workout fruit
Almonds, 10	~70 kcal	~3 g	Portion-controlled fat

Reading this table correctly

Roti and rice are not the enemy; volume is. A single chapati is about 114 kcal, so the damage usually comes from three of them with a heavy sabzi, not from one. Protein is where most Indian plates fall short, so build each meal around a protein anchor and let carbohydrate fill the smaller quarter.

CHAPTER 8

The Invisible Calories: Chai, Namkeen, Oil and Festivals

Most Indian diets do not fail at the dining table. They fail in the gaps: the four cups of chai, the packet of namkeen, the extra spoon of ghee, the mithai at a colleague's desk. These calories are invisible because nobody counts them, and they easily add 400-700 kcal a day.

Habit	Typical calories	Fix
Chai with 2 tsp sugar, 4 cups/day	~200-240 kcal	Cut to 1 tsp, or none; save ~150 kcal
Biscuits with tea, 4-5 pieces	~200 kcal	Swap for roasted chana or a fruit
Namkeen / bhujia, 1 small handful	~150-200 kcal	Pre-portion; never eat from the packet
Extra oil, 2 spoons/day	~180 kcal	Measure with a spoon; cut 10%
One gulab jamun / mithai	~150-300 kcal	One piece, then stop; budget it in

Why liquid and snack calories are the worst offenders

They do not fill you up. A 200-kcal plate of dal and salad keeps hunger away for hours; a 200-kcal cup of sweet chai barely registers and leaves you hungrier later. Liquid sugar and fried snacks are the lowest-value calories in the Indian diet, which is exactly why they are the first thing to cut.

The festival reality

India's calendar is a sequence of food events, and pretending otherwise sets you up to fail. The answer is not to refuse every sweet; it is to plan. Decide in advance which two or three festival days you will enjoy freely, and treat every other day normally. On feast days, fill up on protein and vegetables first, take one serving of the sweet, and walk it off afterwards. One day never undid a week of good eating; a week of 'it's festive season' does.

CHAPTER 9

Move More: The Minimum Effective Dose

You do not need a gym membership, a trainer, or two hours a day. The World Health Organization's global guidelines, which India follows, ask for a modest and specific amount of activity, and most of the benefit comes from simply moving more than you do today.

The WHO target for adults

150 to 300 minutes of moderate aerobic activity per week (a brisk 30-40 minute walk, five days a week, does it), PLUS muscle-strengthening activity for all major muscle groups on 2 or more days a week. Any movement is better than none, and every minute counts.

Why strength training is non-negotiable for Indians

Indians typically have lower baseline muscle mass. In a calorie deficit, the body will happily burn muscle for fuel if you do not give it a reason not to. Strength work is that reason. Two short sessions a week protect the muscle that keeps your metabolism and your shape, so the weight you lose is fat.

A no-gym week that works

Day	Session	Time
Mon	Walk briskly	35 min
Tue	Bodyweight strength: squats, push-ups (wall or knee), lunges, plank	25 min
Wed	Walk briskly	35 min
Thu	Walk + stairs	30 min
Fri	Bodyweight strength: repeat Tuesday with more reps	25 min
Sat	Longer walk or cycle	45 min
Sun	Rest, or a gentle walk	20 min

Never underestimate NEAT, or 'non-exercise activity'. Standing while you work, taking stairs, walking to the market, pacing on phone calls: these can add 200-500 kcal a day for an active person versus a sedentary one, without a single workout.

CHAPTER 10**Sleep and Stress: The Fat-Loss Hormones**

Two levers nobody talks about at the dinner table decide how hungry you feel and where your body stores fat. Get them wrong and even a perfect diet fights you.

Sleep

When you sleep under about seven hours, your body raises ghrelin (the hunger hormone) and lowers leptin (the fullness hormone). You wake up hungrier, you crave sugar and fried food, and you store more fat, especially around the belly. Short sleep also lowers the proportion of weight lost as fat. Aim for seven to eight hours, at a consistent time, with no screens for the last half hour.

Stress

Chronic stress keeps cortisol elevated, and cortisol pushes fat storage toward the abdomen. Indian daily life, with its commutes, work pressure and family responsibilities, supplies plenty of it. You cannot delete stress, but you can blunt it: a ten-minute walk, five minutes of slow breathing, prayer or meditation, a phone call with a friend, or simply a screen-free meal. These are not soft extras; they are part of the fat-loss system.

The evening routine that helps

Aim to finish your last full meal 2-3 hours before bed. Take a 10-minute walk after dinner to lower the post-meal blood-sugar spike. Then wind down without screens. Good sleep is the cheapest fat-loss tool you own.

CHAPTER 11

The Seven Mistakes That Stall Indian Diets

Almost every stalled Indian diet can be traced to one of these seven errors. Fix the one you recognise and progress usually restarts within a week.

- 1 **Too little protein.** The plate is all rice and roti. Add a protein anchor to every meal and the deficit appears on its own.
- 2 **Crash dieting.** Eating 800 kcal a day strips muscle, crashes energy and guarantees a rebound. Use a moderate 300-500 kcal deficit.
- 3 **Liquid calories.** Sweet chai, packaged juices, cold drinks and 'health' smoothies quietly add hundreds of calories that never fill you.
- 4 **Relying on supplements.** Fat burners, detox teas and mass-market protein powders promise much and deliver little. ICMR-NIN explicitly advises against protein supplements for healthy people building muscle.
- 5 **Scale obsession.** Weight swings 1-2 kg a day with water, salt and hormones. Judge progress monthly, and track your waist, not just the number.
- 6 **Skipping meals, then overeating.** One giant late dinner undoes the day. Three steady meals plus one planned snack works better.
- 7 **Zero strength training.** Cardio alone burns calories but lets muscle slip. Two short strength sessions a week change the outcome.

The myth worth burying

A 'normal' BMI does not mean you are metabolically safe. Indians can be metabolically obese at a normal weight. Your waist and your blood sugar tell the truer story. Get a basic screening (fasting glucose, HbA1c, lipids, blood pressure) at least once a year.

CHAPTER 12

When the Scale Stops: Breaking a Plateau

A plateau is not failure; it is arithmetic. As you lose weight, your body needs fewer calories to run, so the same diet slowly stops being a deficit. Add normal water fluctuation and it can look like nothing is happening for two or three weeks.

Before you change anything, rule these out:

- You are weighing at different times of day, or after salty food, or around your period.
- Your 'one spoon' of ghee or oil has quietly grown into two or three.
- Weekend eating is undoing the week. Two big weekend days can erase a 300 kcal daily deficit.
- You have lost fat but gained water from a new workout, masking the change on the scale.

If it is a true plateau, use these levers in order

- 1 Re-measure your waist. If it is smaller, you are still losing fat, even if the scale is stuck.
- 2 Tighten the calorie side slightly: cut 100-150 kcal, ideally from oil, sugar and refined carbs, not from protein.
- 3 Add movement before you cut food further: an extra 2,000 steps a day, or one more strength session.
- 4 Check protein and sleep. Both slip quietly during a long diet, and both stall fat loss.
- 5 Take a planned 'maintenance' week: eat at your new TDEE for 5-7 days. This restores hormones and energy, and often unlocks the next phase.
- 6 If you have been in a deep deficit for months, a short diet break is smarter than pushing harder.

Perspective

A plateau of 2-3 weeks after months of progress is normal and expected. The people who succeed are the ones who adjust calmly instead of quitting or crash-dieting in frustration.

CHAPTER 13

Festival-Proofing: From Navratri to the Wedding Season

Between Navratri, Durga Puja, Dussehra, Diwali, Bhai Dooj, weddings and the New Year, the Indian calendar runs a food marathon from October to February. This is exactly when most people's fat loss dies. Here is how to come out the other side lighter anyway.

The festival strategy

Situation	The move
Navratri fasting	Do not 'compensate' with fried sabudana and potato. Use curd, paneer, rajma, makhana, fruit and roasted peanuts for protein.
Diwali sweets	Choose your two favourite mithai days. One piece each, then stop. Skip the rest without guilt.
Wedding buffets	Fill half the plate with salad and grilled protein first, then take small portions of the rich dishes.
Continuous guests	Keep a protein snack ready: roasted chana, curd, boiled eggs, makhana. Never arrive at a buffet starving.
Travel	Carry roasted chana, nuts and a shaker of plain water. Airport and station food is where diets go to die.

The one rule that saves the season

Decide in advance which specific days are 'free' and which are normal. If you mark two or three celebration days and treat every other day as a normal day, you will hold your weight through the season and often still lose. It is the unbroken month of 'it's festive time' that adds the kilos, not a single celebration.

The 80/20 season

Eighty percent of days normal, twenty percent enjoyed. That is not a compromise; it is the only strategy that survives a real Indian year.

CHAPTER 14**PCOS, Thyroid and Diabetes: The Indian Realities**

For many Indian women and men, fat loss is not a simple calorie equation. Hormones are involved, and ignoring them wastes months. This chapter is education; your diagnosis and treatment belong with a doctor.

PCOS, which affects a large share of Indian women

An ICMR national task-force study of nearly 9,000 women aged 18-40, published in JAMA Network Open, put the prevalence of PCOS at 7.2% by strict criteria and up to 19.6% by the broader Rotterdam criteria. Among the women with PCOS, using Asian cutoffs, 43.2% had obesity, 91.9% had some lipid abnormality, and about a quarter had metabolic syndrome. PCOS is not just a fertility issue; it is a metabolic one.

Thyroid

An underactive thyroid (hypothyroidism) can slow metabolism and make weight gain easier and loss harder, but it is rarely the whole story. If you are persistently tired, cold, constipated, or losing hair alongside stubborn weight, ask your doctor for a thyroid test. Correcting it helps; it does not by itself cause fat loss.

Prediabetes and diabetes

With ICMR-INDIAB finding prediabetes in 15.3% and diabetes in 11.4% of adults, many readers are already on the metabolic spectrum. The good news: the same plan works, with adjustments. Protein at every meal, fewer refined carbs, more fibre, resistance training and better sleep all improve blood sugar. If you take diabetes medication, coordinate with your doctor, because weight loss can change your dose needs.

What to do this week

If you have not had a basic screening in the last year, book one: fasting glucose or HbA1c, a lipid profile, thyroid (TSH) if symptomatic, and blood pressure. Knowledge turns guesswork into a plan.

CHAPTER 15**The GLP-1 Era: What Every Indian Should Know**

You have heard of Ozempic, Wegovy and Mounjaro. India's weight-loss medicine market has exploded, and it is now cheap enough to matter. This chapter explains the landscape honestly, so you can have a good conversation with your doctor rather than a bad one with a WhatsApp seller.

What these medicines are

GLP-1 medicines mimic a gut hormone that slows stomach emptying and reduces appetite and cravings. Semaglutide (Rybelsus, Ozempic, Wegovy) targets the GLP-1 receptor; tirzepatide (Mounjaro, Yurpeak) targets GLP-1 and GIP together and produces more weight loss in trials. They are prescription medicines, not supplements.

What changed in India in 2026

- The semaglutide patent expired in March 2026, and more than 50 generic brands entered the market.
- Generic semaglutide now starts at roughly Rs. 350 per week, a fraction of the originator price.
- Branded options range from about Rs. 5,660 a month (Wegovy starter) to Rs. 18,000-26,000 for higher doses.
- Oral GLP-1 pills (oral Wegovy and Lilly's orforglipron) are expected in India in the first half of 2027.

The honest trade-offs

These drugs work, but they are not a shortcut past nutrition. Indian endocrinologists warn that about a quarter of the weight lost on GLP-1 therapy can be muscle, especially with low protein and no resistance training. Most people regain weight within a year of stopping. Side effects, cost and long-term adherence are real. Used well, with structured nutrition and strength work, they are a powerful tool for the right patient. Used casually, they are an expensive round trip.

The rule

GLP-1 medicines are a doctor's decision, never a self-service purchase. If you use one, treat this book's protein and strength chapters as mandatory, not optional. That is what protects your muscle and your result.

CHAPTER 16

Supplements: What Works and What Is a Waste

India's supplement market sells hope by the kilo. Here is a short, honest filter. Remember that ICMR-NIN advises getting protein from food and avoiding protein supplements for healthy people building muscle.

Supplement	Verdict	Why
Whey protein	Optional	Only if you genuinely cannot hit protein from food. It is a convenience, not a fat-burner.
Creatine monohydrate	Worth considering	One of the best-evidenced supplements for strength and muscle. Cheap and safe for most.
Vitamin D	Often needed	Deficiency is very common in India. Test, then supplement on your doctor's advice.
Multivitamin	Situational	Fills gaps if your diet is poor; not a substitute for real food.
Fat burners / detox teas	Skip	Little to no evidence, sometimes harmful, often expensive.
Meal-replacement shakes	Skip for most	Expensive, less filling than dal and curd, and easy to abandon.

The bottom line

No pill has ever outperformed a plate of dal, curd, eggs, vegetables and roti eaten in the right amounts. Spend your money on good food and, if anything, on creatine and a vitamin D test. Everything else is marketing.

CHAPTER 17**The 12-Week Plan**

This is where everything comes together. Twelve weeks, three phases, one clear routine. If you do nothing else in this book, do this.

Set your numbers first (Week 0)

- 1 Weigh yourself and measure your waist. Photograph yourself if you can.
- 2 Calculate your TDEE (Chapter 3) and subtract 400-500 kcal. Set your protein target at 1.2-1.5 g per kg.
- 3 Book a basic health screening if you have not had one in a year.
- 4 Clear your kitchen of the invisible-calorie traps: sweet biscuits, namkeen packets, sugary drinks.

Phase 1: Weeks 1-4 - Build the habits

- Use the Indian plate method at lunch and dinner: half vegetables, a quarter protein, a quarter smart carb.
- Add a protein anchor to breakfast every day.
- Cut chai sugar to one teaspoon or none.
- Walk 30-40 minutes, five days a week. Two short strength sessions a week.
- Sleep 7-8 hours. No screens for the last 30 minutes.

Phase 2: Weeks 5-8 - Tighten and strengthen

- Keep the plate method; make sure protein hits target every single day.
- Add one more strength session or increase reps.
- Cut oil and fried snacks further; use a measuring spoon.
- Start tracking waist weekly alongside weight.
- Handle your first festival with the two-free-days rule.

Phase 3: Weeks 9-12 - Consolidate

- If progress has slowed, re-measure your waist and re-calculate your TDEE, then adjust (Chapter 12).
- Hold protein and strength work steady; this is where muscle is protected.
- Plan your maintenance strategy: the habits you will keep for life, not just for twelve weeks.
- Re-screen your blood sugar and lipids if you started with abnormal values.

What success looks like

A sustainable 0.5 kg a week, waist down several centimetres, more energy, better sleep, and clothes that fit differently. That is the outcome this plan is built for, and it is the outcome that lasts.

CHAPTER 18

Meal Templates and Worksheets

Use these as starting points, not rules. Swap any item for a similar one you actually enjoy, keep the protein anchor, and adjust portions to your calorie target.

Vegetarian day (about 1,600-1,800 kcal)

Meal	Example	Protein
Breakfast	2 besan chilla + 1 bowl curd + 1 fruit	~18 g
Mid-morning	Roasted chana, small handful	~7 g
Lunch	2 rotis + 1 bowl dal + big sabzi + salad + curd	~22 g
Evening	Sprouts chaat or paneer cubes	~12 g
Dinner	1 roti or small rice + rajma/paneer + vegetables	~20 g

Non-vegetarian day (about 1,600-1,800 kcal)

Meal	Example	Protein
Breakfast	3-egg omelette + 1 slice toast + 1 fruit	~22 g
Mid-morning	Curd or buttermilk	~5 g
Lunch	2 rotis + chicken curry (lean) + salad	~28 g
Evening	Boiled egg or roasted chana	~7 g
Dinner	Grilled fish/chicken + vegetables + 1 roti	~30 g

Your weekly tracker

Copy this into a notebook. Fill it in every Sunday morning, under the same conditions.

Week	Weight (kg)	Waist (cm)	Protein hit?	Walks done	Strength done	Notes
1						
2						
3						
4						
6						
8						
10						
12						

Keep it boring

The people who lose fat and keep it off are not the ones with the most interesting diet. They are the ones who repeat a simple, protein-rich, mostly-home-cooked routine for months. Boring is the strategy.

CHAPTER 19**Eating Out, Ordering In and Hostel Food**

Most Indian diets do not collapse at home; they collapse at a restaurant, a Swiggy order, or a hostel mess. You can eat out and still lose fat. You just need a few default moves.

Restaurant and takeaway rules

Situation	Default move
North Indian mains	Choose tandoori/grilled over creamy curries. Dal makhani, butter chicken and malai kofta are calorie bombs.
Breads	One or two plain rotis, not butter naan or paratha. Naan can be 250-350 kcal each.
Starters	Tandoori paneer, chicken tikka or soup over fried pakora and spring rolls.
South Indian	Idli, plain dosa or uttapam with sambar; go easy on coconut chutney and ghee roast.
Chinese	Plain steamed or gravy over fried rice and Manchurian, which are oil-heavy.
Beverages	Water, buttermilk or fresh lime without sugar. Skip sweet lassi and cold drinks.

Ordering in without sabotaging the week

- Order one grilled protein and one vegetable dish, plus a shared bread or rice. Eat protein first.
- Never order while starving. Have a glass of water and a handful of chana before the food arrives.
- Skip the free dessert upsell. It is usually 300-500 kcal you did not plan for.
- If you order a curry, ask for less oil or cream, and spoon off the surface oil.

Hostel, mess and canteen survival

You often cannot choose the menu, so choose the portion and the additions. Take more of the dal, curd, egg and vegetable and less of the fried item and the sweet. Keep your own protein in your room: roasted chana, peanuts, curd if you have a fridge, boiled eggs, or a shaker of plain milk. These small additions close the protein gap that mess food almost always leaves open.

CHAPTER 20

The Home Workout Library

You need no equipment and no gym for the strength work that protects your muscle. Here is a complete beginner routine you can do in a bedroom or a living room. Do two sessions a week, on non-consecutive days, and progress by adding reps or slowing the movement.

Exercise	What it does	Beginner dose
Bodyweight squat	Legs, glutes, core	2 sets x 10-12 reps
Wall or knee push-up	Chest, arms, shoulders	2 sets x 8-12 reps
Reverse lunge	Legs, balance	2 sets x 8 per leg
Glute bridge	Glutes, lower back	2 sets x 12 reps
Plank	Core	2 sets, hold 20-30 sec
Bird-dog	Core, lower back	2 sets x 8 per side
Chair triceps dip	Arms	2 sets x 8-10 reps
Standing calf raise	Calves	2 sets x 15 reps
Superman hold	Lower back	2 sets, hold 15-20 sec
Step-up on a stair	Legs, glutes	2 sets x 10 per leg
Wall sit	Legs, endurance	2 sets, hold 20-30 sec
Dead-bug	Deep core	2 sets x 8 per side

How to progress without a gym

- 1 Weeks 1-2: learn the movements with 2 sets each, focusing on form over speed.
- 2 Weeks 3-6: add a third set, or slow each rep to a 3-second lowering.
- 3 Weeks 7-12: add reps, add a resistance band or a filled water bottle, and reduce rest between sets.
- 4 Always warm up with a 5-minute walk and finish with a gentle stretch.

Walking is the base

Strength protects muscle; walking burns calories and is easy to sustain. Aim for 7,000-10,000 steps on most days, and treat your two strength sessions as the non-negotiable extras that keep the weight loss coming from fat, not muscle.

APPENDIX A

A 7-Day Vegetarian Plan

Adjust portions to your calorie target. Every day lands roughly 1,500-1,700 kcal with 80-100 g protein. Swap freely, keeping a protein anchor in every meal.

Day	Breakfast	Lunch	Snack	Dinner
Mon	2 besan chilla + curd	2 roti + dal + sabzi + salad	Roasted chana	Paneer bhurji + 1 roti + veg
Tue	Poha + 1 bowl curd	1 rice + rajma + salad	Buttermilk + nuts	Soya chunk curry + 1 roti
Wed	2 idli + sambar	2 roti + chole + veg	Sprouts chaat	Dal + 1 roti + lauki sabzi
Thu	Vegetable dalia + curd	1 rice + moong dal + veg	Curd + fruit	Tofu/paneer + veg stir-fry
Fri	2 moong dal chilla	2 roti + mix dal + salad	Roasted chana	Rajma + 1 roti + salad
Sat	Upma + curd	1 rice + sambar + veg	Nuts + fruit	Palak paneer + 1 roti
Sun	2 eggs-free uttapam + sambar	Veg pulao (small) + dal + raita	Buttermilk	Chole + 1 roti + veg

APPENDIX B

A 7-Day Non-Vegetarian Plan

The same rules apply: protein first, vegetables second, carbohydrate last. Portions adjust to your calorie target.

Day	Breakfast	Lunch	Snack	Dinner
Mon	3-egg omelette + toast	2 roti + chicken curry + salad	Boiled egg	Grilled fish + veg
Tue	Egg bhurji + 1 roti	1 rice + fish curry + veg	Curd	Chicken tikka + salad
Wed	2 eggs + fruit	2 roti + egg curry + veg	Roasted chana	Chicken soup + 1 roti
Thu	Omelette + curd	1 rice + chicken + salad	Nuts	Grilled chicken + veg
Fri	3 eggs + toast	2 roti + fish + veg	Boiled egg	Egg curry + 1 roti
Sat	Egg dosa + sambar	1 rice + chicken curry + veg	Curd	Tandoori chicken + salad
Sun	Omelette + fruit	2 roti + mutton (lean, small) + veg	Buttermilk	Fish + veg stir-fry

APPENDIX C

How to Read an Indian Food Label

Packaged food is where 'healthy' claims go to hide. The ICMR-NIN guidelines ask Indians to read the label and make informed choices. Here is how to do it in thirty seconds, using the FSSAI-mandated format.

What to look at	What it tells you	Red flag
Ingredients list	Listed in descending order by weight	Sugar, maida or palm oil near the top
Per 100 g column	Lets you compare two products fairly	A 'small serving' that hides high calories
Added sugar	Total sugar minus natural	More than 5% of your daily calories
Fat and saturated fat	Total fat and the saturated share	Saturated fat high relative to total
Sodium / salt	Hidden salt in snacks and sauces	High per 100 g; you need under 5 g salt a day
% RDA per serving	Share of daily needs per serving	A 'single serving' that is really two
Front-of-pack claims	'Sugar-free', 'whole grain', 'high protein'	Cross-check the panel; claims can mislead

Three habits that beat any claim

- Compare per 100 g, never per serving, when choosing between two packs.
- If sugar or refined flour is in the first three ingredients, treat it as a treat, not a staple.
- If the ingredient list is longer than your shopping list, it is probably ultra-processed. ICMR-NIN advises minimising ultra-processed foods.

The simplest rule

Foods without labels are almost always the better choice. Dal, vegetables, curd, eggs, fruit and fresh roti need no ingredient list. The more of your plate that comes without packaging, the less you have to read.

Quick-Start Cheat Sheet

Print this page or photograph it. If you remember nothing else, remember this.

The seven rules

1. Eat protein at every meal (target 1.2-1.5 g per kg per day).
2. Use the Indian plate: half vegetables, a quarter protein, a quarter smart carb.
3. Cut invisible calories: chai sugar, biscuits, namkeen, excess oil.
4. Walk 30-40 minutes, five days a week; strength train twice a week.
5. Sleep 7-8 hours; manage stress.
6. Track your waist, not just the scale, once a week.
7. Plan your festivals: two or three free days, everything else normal.

0.5 kg**PER WEEK**

sustainable fat loss

2**STRENGTH DAYS**

protect your muscle

90 / 80 cm**WAIST LIMIT**

men / women, Indian cutoffs

The one habit to start tomorrow

Add a protein anchor to your breakfast and cut your chai sugar to one teaspoon. Two small changes, done daily, will start the fat loss before you touch anything else.

Frequently Asked Questions

Do I have to give up rice and roti?

No. Carbohydrate is not the enemy; portion and context are. Keep one to two rotis or a small bowl of rice, but build the rest of the plate from vegetables and protein. Most Indian diets fail from too much refined carb and too little protein, not from rice itself.

How fast will I lose weight?

At a 400-500 kcal daily deficit, expect roughly 0.5 kg of fat per week, with normal water fluctuation on top. Weigh monthly for the true trend and track your waist weekly.

Is intermittent fasting necessary?

No. Intermittent fasting works because it helps you eat fewer calories, not through any magic. If a 16:8 window suits your routine and you still hit protein and calories, it is a fine tool. If it makes you overeat at night, skip it.

Do I need protein powder?

For most people, no. ICMR-NIN advises getting protein from foods and avoiding supplements for healthy people building muscle. Whey is a convenience if you genuinely cannot hit your target from dal, curd, paneer, eggs and chicken.

I have PCOS / thyroid. Will this work for me?

The principles still apply, with adjustments, and you should work with your doctor. Protein, fibre, reduced refined carbs, strength training and sleep all help PCOS and metabolic health. Correcting a thyroid problem helps too, but is rarely the whole story.

Should I consider a weight-loss medicine like Ozempic or Mounjaro?

That is a decision for you and a qualified doctor, not a self-service purchase. These are prescription medicines with real trade-offs, including muscle loss and weight regain after stopping. If you use one, protein and strength work become essential.

How do I keep it off after twelve weeks?

You do not go back to old habits; you shift to maintenance. Keep the plate method, keep protein high, keep two strength sessions a week, and allow your calories to rise to your new maintenance level. The habits are the result.

Sources and Further Reading

Every statistic in this book is drawn from the following official or peer-reviewed sources, current as of October 2026.

- Ministry of Health and Family Welfare, Government of India - National Family Health Survey 6 (NFHS-6), 2023-24, released 29 May 2026 (via IIPS and PIB).
- ICMR-INDIAB national cross-sectional study, The Lancet Diabetes and Endocrinology, 2023 - prevalence of diabetes, prediabetes, obesity and abdominal obesity.
- ICMR-National Institute of Nutrition - Dietary Guidelines for Indians 2024 (17 guidelines on protein, sugar, fat and cereals).
- ICMR-NIN - Nutrient Requirements for Indians: RDA and EAR 2020 (protein RDA 0.83 g/kg/day; 1 g/kg for cereal-based diets; 3:1 cereal-to-pulse ratio).
- World Health Organization - Guidelines on Physical Activity and Sedentary Behaviour, 2020 (150-300 minutes plus 2 days of muscle strengthening).
- Misra A. et al. - Waist circumference cutoff points for Asian Indians, International Journal of Obesity, 2006 (action levels 90 cm men, 80 cm women).
- WHO Asia-Pacific BMI classification for Asian populations (overweight from 23, obesity from 25).
- Indian endocrinology consensus on nutrition alongside GLP-1 therapy, published in Obesity Pillars, 2026 (protein 1.2-1.5 g/kg during weight loss).
- ICMR national task-force study on PCOS among Indian women, JAMA Network Open, 2024.
- ICMR-NIN - Indian Food Composition Tables (IFCT) 2017, used for the calorie and protein values in Chapter 7.
- Kantar - India in Search 2026 report (health and wellness searches at 52 million a month, up 10% year on year).
- Pharmarack and press reporting on India's GLP-1 and anti-obesity market, 2025-2026 (pricing and availability of semaglutide and tirzepatide).

A final note

This book is general education, not medical advice, and no book can replace a qualified doctor who knows your history. Use it to eat better, move more, sleep well and ask your doctor smarter questions. That is what it is for.